



Nursing Practice Guideline

Nursing Documentation

Reference #: NPG25

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation-Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to nursing documentation. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☐
Other:	☐		

Table of Contents

Introduction	1
Risk Statement	1
1.0 General Principles	2
2.0 Admission and Assessment.....	3
3.0 Nursing Care Plan	7
4.0 Fluid Balance Monitoring	7
5.0 Physiological Observations and Clinical Escalation.....	8
6.0 Transfer between Wards/Departments	10
7.0 Discharge	10
Related Documents	12
References	12
Authorisation & Version Control.....	13
8.0 APPENDIX 1 - Weighing Patients.....	14

1.0 General Principles

1.1 Safety Information

The use of abbreviations must be avoided.¹

All entries are to be legible and made in black or blue ink. Pharmacists may use purple ink to differentiate their notation on the medication chart.

No space is to be left between entries. If this is unavoidable, then a diagonal line should be drawn across the blank space to ensure additional entries cannot be made.

Where an undergraduate student is required to make a notation in the medical or nursing records, the entry will be reviewed and countersigned by their supervisor / preceptor.^{1,2}

Entries made by Assistants in Nursing (AIN's) must be countersigned by a registered / enrolled nurse^{2,3}.

The medical record is a confidential record and should not be removed from the clinical area for documentation purposes and accessed only by relevant personnel.

1.2 Nursing Management

A written report on each patient is essential at least once every 24 hours by medical and nursing staff and further reports throughout the 24-hour period are required for additional or significant information:

- The designated shift for 24hrs entry is the morning shift. If there is no change in the delivery of nursing care from what is documented and the patients' condition has not changed, there is no requirement for afternoon / night shift to make an entry in the inpatient notes, provided the Comprehensive Care Plan has been signed for care delivered.
- The patient identification (full name, address, DOB, Unique Medical Record Number (UMRN)) is present on all documentation.
- Every entry must have the date and time and be followed by the author's full signature, printed last name (surname) and designation.
- Entries should, where possible, be written contemporaneously. If this is not possible, the entry in the notes should state that it is a "late entry / addit" and made as soon as practicable with the date and time of the addition.

Corrections must be made with a single line through the error, write "written in error" alongside, and the corrected text entered nearby. Each correction must be dated and signed. Correction fluid / tape must never be used. Only document care after it has been given.

- All inpatient documentation is to be recorded using a "systems" approach ^{4,8} at least once every 24hrs.
 - CNS (Central Nervous System)
 - CVS (Cardiovascular System)
 - RESP (Respiratory Function)
 - RENAL (Renal Function)
 - GIT (Gastrointestinal)
 - Metabolic (Endocrine)
 - Integument / Wounds
 - Musculoskeletal
 - Social
 - Plan
- It is a requirement on every shift that nursing staff record and sign the care given for each patient on the Comprehensive Care Plan.

- Procedure specific clinical pathways are used in select clinical areas (e.g., Radiology Day Unit, Day Procedure Unit, Cardiothoracic surgery, and Coronary Care Unit). These clinical pathways outline the expected care path of patients post-operatively or post procedure.
 - If the patient condition or care deviates from the prescribed clinical pathway a variance note is used to document patient care and management.
 - If the patient deviates greatly from the pathway (e.g., the expected plan of care is no longer appropriate) or their admission extends beyond the expected length of stay, a written report and plan for patient care must be transferred to integrated inpatient notes and a standard hospital comprehensive care plan implemented.
- Care Plan for the Dying Person is used for end-of-life care.

2.0 Nursing Admission and Assessment

2.1 Safety Information

Assessment forms must include the admitting nurses' printed name, signature, designation and the date of admission.

SCGH: Patients identified as >120kg require a weight 'W' code or >150kg 'W+' code must be documented on the journey (white) board and on i-Clinical Manager nursing clinical handover.

OPH: All larger and/or higher BMI patients will be risk assessed for safe admission to OPH utilising the Assessment Tool for Larger Patient Admissions. The larger patient is defined as anyone who has limitations in health due to their physical size, lack of mobility, due to their size, weight or high body mass index

Patient identification details / label must be present on all clinical documentation.

Patient identification must be checked when matching patient's identity to care, therapy and services.

2.2 Nursing Management

SCGH Nursing Admission Assessment

- Patients attending the Pre-Admission Clinic will have the Nursing Admission Assessment commenced prior to admission.
- On the day of admission information within the Nursing Admission Assessment will be reviewed and updated.
- All overnight /multiday admission areas (Emergency, Direct and Booked) must commence the Nursing Admission Assessment - with the following documents commenced on admission to inpatient wards (including Short Stay Unit):
 - Nursing Admission Assessment Form and Comprehensive Care Plan
 - Observation and Response Chart
 - The Intensive Care Unit (ICU) and) utilise integrated risk assessments within the ICU Nursing Admission Assessment and Care Plan, and ICU Flow Chart.

OPH Nursing Admission Assessment Non-Surgical

- On admission to inpatient ward, the following documents are commenced
 - Nursing Admission Assessment Form and Comprehensive Care Plan
 - Observation and Response Chart

OPH Surgical Services

- Patients attending the Pre-Admission Clinic will have the OPH Patient Health history and Preadmission Clinic assessment completed prior to admission.

- On the day of admission information within the Nursing Admission Assessment– will be reviewed and updated as required.
- All overnight /multiday admission areas must commence the Nursing Admission Assessment - with the following documents commenced on admission to the inpatient ward
 - Nursing Admission Assessment and Comprehensive Care Plan
 - Observation and Response Chart

Ensure all other charts are commenced as clinically indicated.

2.3 Procedural Information

All sections on the Nursing Admission Assessment are to be completed. This represents an assessment of what the patient was like **prior** to/on admission to hospital.

Falls risk screening should to be completed and actioned within 6 hours of admission. All other risk assessments should be completed and actioned within 8 hours of admission. Interventions and preventative strategies must be documented in the Comprehensive Care Plan.

Patient Information

Record estimated date of discharge (EDD) if known. This should be discussed with the treating team and established within 12 hours of admission. If any discharge concerns are anticipated the appropriate allied health referrals are to be initiated.

Determine patient's preferred name and write this on the patient bedside communication board and other relevant locations.

Ascertain who the patient wants involved in their care decision making and ascertain what their goals are i.e., what the patient hopes to achieve during this admission.

Identify what matters to the patient.

Determine patient perception of reason for admission and record any relevant history.

Alert / Allergies

Record any allergies or implantable devices.

If Drug or Food Allergies are known; on admission ensure the patient has a RED hospital identity band in situ, also ensure an Adverse drug Reaction sticker is applied to WA Hospital Medication Chart (WAHMC). Add food allergies and intolerance to the automated menu system (AMS).

Advanced Care Planning

Determine if the patient has an Advanced Health Directive, Enduring Power of Attorney/Guardian or a previous Goals of Patient Care Form with section 4 completed.

Ensure a copy is obtained for the medical record

Consumer

Determine current living arrangements.

Discuss cultural health beliefs and the impact on health care. Consider referral to pastoral services. Aboriginal Hospital Liaison Officer.

Determine if the patient utilises community services and if these services are aware of their admission. Identify early the need for Allied Health Referrals.

Communication

Determine if the patient understands and speaks English.

If the patient is from a Culturally and Linguistically Diverse (CALD) background, document their primary language and determine the need for interpreter services during admission. Ensure a language alert sticker is placed on relevant documentation.

Identify and document use of aids. Record in the Property section if glasses / hearing aids have been brought into the hospital.

Infection Control

Determine patient status in relation to known or suspected infection control issues / micro alerts.

Complete required screening swabs and institute contact precautions as indicated.

Hazardous / Antineoplastic Medication

Determine if current medications have been taken within 7 days and implement cytotoxic precautions and signage as indicated.

Mental Health

If a current mental health and/or wellbeing need / risk is identified ensure the treating team is aware to refer to psychiatry / social work as required.

Cognition

Determine if the patient has a known Cognitive Impairment.

Establish baseline cognition by completing 4AT as per criteria. Inform Medical staff of abnormal 4AT score.

Incorporate any patient information provided by family/care provider into care planning.

Wound

Commence Wound Assessment and Care Plan if wound present

Use Waterlow tool and clinical judgement to screen for pressure injury risk and implement prevention strategies.

Substance Use

Consider referral to Drug and Alcohol Liaison Service; discuss with Medical Officer.

Commence Substance Withdrawal Chart if required.

Behaviour

Determine if there is a documented history of aggression or violence in the inpatient medical notes or a WebPAS flag and review requirement for an Aggression, Prevention, and Intervention Risk Assessment.

NB: This is not a question to ask of the patient, gain this information from previous documentation

Pain

Assess history of pain, acute and chronic pain. Provide education on the use of pain scores in hospital.

Activities of Daily Living

Determine function and mobility prior to admission to hospital.

Record use of aids and level of assistance required.

Nutrition

Record weight and height on admission. Subsequent weight measurement is to be performed weekly as a minimum or more frequently according to patient condition and recorded on the ORC and WA Hospital Medication Chart (WAHMC).

Assess for the need for bariatric equipment. If the patient is identified as >120kg or >150kg, consider individual equipment needs, refer to SCGH Bariatric Patient Admission Flowchart. / OPH Assessment tool for the larger patient for admission

Assess for special dietary requirements and fluid restrictions and enter into the Automated Menu System (AMS).

Calculate Malnutrition Screening Tool (MST) Score and enter score into AMS to ensure that patients identified at risk of malnutrition are not further compromised due to weight loss during hospitalisation. Report any unexplained weight loss to the medical officer in charge of care..

Elimination

Problems with urinary or bowel elimination must be documented, including the date of bowels last opened and time of last void. Indicate if continence chart commenced.

All patients are to have urinalysis completed within 24 hours of admission and at intervals as determined following clinical assessment.

Patients Own Medication (POM)

Record if patient has taken their usual medication on day of admission and secure POM appropriately on ward. The correct handling, documentation (e.g., recorded in dedicated patients own S8 /S4R register) and storage of S8 and S4R medicines must be followed.

POM should not be sent home with the patient's family/carer as they are useful for medication reconciliation, may be required for administration. POM should be reconciled by a clinical pharmacist prior to discharge.

Property

Document if the patient has property with them on admission.

Ensure patient is informed that the Hospital takes no responsibility for property kept on the ward and encourage them to send their property/valuables home with a relative or representative.

Property can be stored securely with

- SCGH: the Property Office.
- OPH: the Nurse Manager, Patient Flow or the After-Hours Clinical Nurse Manager (AHCNM).

The Patient Property Book must be completed if property is deposited with either.

Ward Orientation

Provide ward orientation- ensure patients are aware of available facilities in ward and how to use call bell.

Inform patients of the free Patient Information Channel which provides information on hospital routines such as meals times and information on Patient Safety.

Provide information on Aishwarya Care Call

Any additional Concerns / Information

Ask if the patient has any other concerns that may not have been addressed during the admission assessment. Provide written information where appropriate.

Risk Screening

All risk screens must be completed on admission within the below timeframes

- Cognitive Risk Screen (as appropriate): within 8 hours of admission

- Pressure Injury Assessment and Malnutrition Risk Screen (MST): within 8 hours of admission
- Falls Risk Screen: within 6 hours of admission.
- Interventions and ongoing preventative measures for all identified risks should be recorded in the Comprehensive Care Plan.

3.0 Comprehensive Care Plan

3.1 Safety Information

Record of care giver to be signed each shift.

3.2 Nursing Management

- Nurse must print / or stamp their full name, designation and sign the front of the Comprehensive Care Plan / or Clinical Pathway.
- The Comprehensive Care Plan should be updated to reflect a change in a patients care.
- The comprehensive care plan will be maintained throughout the patients stay in hospital
- Specimen collections (e.g., VRE, MRSA screening, sputum, MSU) should be documented; note this information is also available on i-Clinical Manager.
- Bedside clinical handover – every patient, every shift, every time refers to the clinical transfer of responsibility and accountability. The handover is required to occur between nursing staff, involving the patient and carer where clinically appropriate, using the iSoBAR process. Nursing staff are to sign the Care Plan, clinical pathway, or designated ICU flowchart on completion of bedside handover.

3.3 Procedural Information

Comprehensive Care Plan

1. Assess individual care requirements each shift, updating / amending each section of the Comprehensive Care Plan to reflect patient care provided. Care provided during the shift must be signed for on each shift.
2. If a particular element was not provided, a line is to be drawn in the box to show acknowledgement of the element – 

Clinical Pathway

1. The 'Record of Care Giver' of a clinical pathway must be signed once only by nurses providing care. The record of care must be signed on each shift.
2. If patient care deviates from the prescribed clinical pathway then a variance note is to be documented in the inpatient notes.
3. If the variance of patient care needs continues longer than 24 hours, or the patient admission extends beyond the expected length of stay, then documentation of nursing care must be transferred onto a Comprehensive Care Plan.
4. Individualised needs and individualised risk management strategies should be documented on the Comprehensive Care Plan if they are not captured on the clinical pathway.

4.0 Fluid Balance Monitoring

4.1 Safety Information

Fluid Balance Chart is to be commenced for the following clinical situations:

- Actual or potential dehydration / fluid overload
- On commencing intravenous or subcutaneous fluid replacement therapy

- Actual or potential acute illness, sepsis,
- Routine post-operative management
- Medical conditions precipitating fluid restrictions / imbalances (e.g. heart disease, renal disease, sepsis)
- Diarrhoea
- Vomiting
- Intravenous & subcutaneous infusions
- Enteral tube – feedings +/- aspiration/drainage
- Invasive drains (wound drain)
- Urinary catheterisation
- If there is doubt over fluid status

The Fluid Balance Summary is to be used in conjunction with all fluid balance charts to record the 24hr summary. The patient trend for +/- balance is to be calculated and transferred to the Fluid Balance Summary chart daily for consideration when making all clinical decisions. Note: fluid balance charts are not routinely filed on discharge.

4.2 Nursing Management

- Urinalysis is to be documented on the observation response chart
- The volume of blood products is to be recorded on fluid balance chart.
- All fluid intake (oral, IV fluids, IV Abs, IV meds) is to be recorded in quantifiable amounts e.g., mL (do not document “sips”).
- All output (vomit, NGT aspirate, bowels/stoma, urine & drains) are to be recorded in quantifiable amounts i.e., mL (weigh continence pads e.g., 100g = 100mL, utilise bottles / bedpan for accuracy).
- Cumulative totals are to be recorded at the end of each shift and the 24hr total is transferred to the Fluid Balance Summary daily at midnight.
- Increasing patient involvement in recording own input / output may result in more accurate fluid balance.
- Any insensible loss which is not measurable should be documented in the inpatient notes (e.g., excessive sweating, increased respiratory rate).

4.3 Procedural Information

1. Fluid balance chart should be used in conjunction with clinical assessment, core physiological observations, body weight and blood results to assess fluid status.
2. Consider cessation of fluid balance chart when:
 - a. The reason for commencement is resolved,
 - b. End of life care.
3. Patients must be assessed thoroughly before making the decision to cease fluid balance chart.

5.0 Physiological Observations and Clinical Escalation

5.1 Medical Requirements

Time Limited Post Procedural Modifications table can only be completed by the Registrar / Consultant of the procedural team. (i.e.: Anaesthetist or Proceduralist)

Clinical Management Plan for Altered Parameters table can only be completed by the Registrar / Consultant of the treating team following medical review.

Documentation on the Observation Response Chart in either the Time Limited Post Procedural Modifications table or Clinical Management Plan for Altered Parameters table (as appropriate) must be completed for adjusted observation parameter ranges to be valid.

Frequency of physiological observations can be decreased by the Registrar / Consultant and changes must be clearly documented in the inpatient notes.

5.2 Safety Information

Medical and nursing staff must document the management plan for monitoring physiological observations, treatment, and escalation of the deteriorating patient at the time of admission. The outcomes of the management plan must be documented in the patient medical records.

Clinicians **must** respond to abnormal physiological observations by following the escalation and action required.

The structured iSoBAR process must be used for all clinical handover as per SCGOPHCG Clinical Handover- Communicating for Safety Policy.

5.3 Nursing Management

- Nursing staff must complete and document a full set of core physiological observations on all inpatients (at least every 8 hours) and in the following situations:
 - When a patient is experiencing, or at risk of experiencing, an episode of acute deterioration,
 - At time of admission or initial assessment,
 - Prior to inter- or intra-hospital transfer.
- The six core physiological observations include:
 - Respiratory rate
 - Oxygen saturation
 - Heart rate
 - Blood pressure
 - Temperature
 - Level of consciousness (AVPU)
- Glasgow Coma Scale (GCS) should be completed and documented at a frequency determined by the home team if required.
- The ORC is not used in the Intensive Care Unit and Emergency Department for documentation of routine vital signs. Prior to transfer to general wards, observations are to be recorded on the ORC and all escalation criteria is to be followed.
- Theatre and procedural departments are not required to document intra-procedural observations on the ORC, although the last set of observations prior to transfer to an inpatient ward are to be documented on the ORC and the escalation criteria followed.
- Levels of clinical escalation include:
 - Increased Surveillance
 - Senior Nurse Review
 - Medical Review
 - Medical Emergency Team
- Response Criteria and/or Actions required are mandatory.
- Patients requiring nursing review are to have the Senior Nurse Review sticker completed and inserted in the patient's inpatient notes.
- Patients requiring medical review are to have the iSoBAR Escalation to Medical Review sticker) completed and inserted in the patient's inpatient notes.

5.4 Procedural Information

1. Document the date the ORC was commenced.
2. Urinalysis must be documented on or within 24hrs of admission on the ORC.
3. Bowel activity must be recorded daily. If altered activity – consider bowel assessment chart.

4. Pulse oximeter or electronic blood pressure measuring devices are not to be used to determine pulse rate. The radial pulse should be palpated to assess rate and rhythm.

6.0 Transfer between Wards/Departments

6.1 Safety Information

To allow successful communication of information from one team, one shift and one ward to the next, plans **must** be documented. All documentation must be thorough, clear and concise, contain easily understood words, with minimal abbreviations.

All clinical handovers must be supported by appropriate endorsed SCGOPHCG documentation when indicated.

Documentation must be completed utilising the iSoBAR process.

6.2 Nursing Management

- **Temporary clinical handover / transfer between interventional / procedural departments**
 - A Departmental specific Checklist is completed as per SCGOPHCG Clinical Handover- Communicating for Safety Policy: refer to Clinical Handover Escort Matrix.
 - All relevant clinical documentation should be sent with the patient to ensure continuity of observation and clinical management.
- **Permanent clinical handover / transfer between wards (within SCGOPHCG)**
 - All details are to be entered into iCM, and bed allocation through Enterprise Bed Management (EBM) and the nurse is to document time of transfer in the medical record.
 - Patients transferred between clinical specialty and / or clinical inpatient departments require the completion of the inpatient Nursing Handover by the transferring ward nurse and escorted with the medical record and patient to the receiving ward. This includes external SCGOPHCG wards (e.g., South Perth Hospital, Woods Ward HPH)
 - The nurse receiving the patient is to sign the inpatient Nursing Handover document acknowledging the patient and handover has been received. If the patient is transferred to a second location within the same shift, the original document can be used with any additional information added and countersigned by the transferring nurse.

7.0 Discharge

7.1 Medical Requirements

All multiday patients discharged from hospital (alive or deceased) must have a complete and accurate medical discharge summary completed by the medical staff member authorizing discharge, or a member of the medical team.

All inpatients who are likely to be discharged from SCGH /OPH the following day without further medical review must have criteria for discharge documented.

Medically completed discharge summaries are to be generated using the site specific approved electronic system and include details of:

- Primary and other relevant diagnoses,
- Treatment course to date including relevant procedures, relevant diagnostic test results and test results pending
- Specific recorded blood transfusion if occurred during hospital stay,

- Discharged medications including changes to medications,
- Pressure injury, falls and infectious risk assessments,
- Outstanding outpatient and medical appointments,
- Ongoing and follow-up plans with responsibilities assigned to specific professions.

Electronic Discharge summaries must be completed for each patient on discharge.

7.2 Safety Information

There will be an estimated discharge date (EDD) written on the journey (white) board and patient communication board and this will be reviewed daily by the shift coordinator in consultation with the medical team. The EDD is entered on webPAS by the ward clerk.

The patient's next of kin / carer are to be informed of the plan for discharge/transfer prior to transfer to another facility and this communication should be documented in the inpatient notes.

If a patient is requesting to discharge against medical advice the patient must be requested to complete the Discharge Against Medical Advice (DAMA) form. If the patient refuses to sign, a member of nursing or medical staff should sign the relevant section of the form.

7.3 08Nursing Management

SCGH only:

- Discharge of patients must be facilitated through the Discharge Ward unless clinically inappropriate. If the patient is transferred to the Discharge Ward and requires completion of care, it is the inpatient ward nurses' responsibility to complete the discharge entry, including any ongoing care instructions to Discharge Ward as well as signing the Discharge/Transfer Checklist.

SCGH & OPH

- Patients with a Pressure Injury (present on admission or hospital acquired) or at risk of developing Pressure Injuries must be assessed prior to discharge or transfer to another health care facility. The outcome of this assessment must be communicated to the health care facility / provider/ carer taking over care.
- Patients identified as at risk of falls during admission must receive falls prevention education. This should be re-iterated to patient or carer on discharge.
- Notification to health care providers regarding the patient's infectious risk for example micro-alert status, current transmission-based precautions must be included in the transfer and discharge documents.
- The Discharge/Transfer Checklist is attached to the discharge envelope for patients returning home or, Discharge/Transfer Checklist and Nursing Transfer Summary for patients being transferred to another facility (hospital or residential care).
- As items are prepared, the Discharge/Transfer Checklist must be marked and the items placed in the envelope. A copy of the electronic Discharge Summary is to be placed in the discharge envelope provided to the patient at the time of discharge and original filed in the medical record.
- The discharging nurse will sign the Discharge/Transfer Checklist and document the destination and time of discharge.
- The Discharge/Transfer Checklist envelope must be signed by patient / carer and on completion, the original 'top' copy should be removed from the envelope and retained in medical records before the Discharge/Transfer Checklist envelope is given to the patient / carer.

- Other requirements / documents for transfer / discharge should be clarified with the facility accepting the patient transfer.
- Notification to health care providers regarding the patient's infectious risk for example micro-alert status, current transmission-based precautions must be included in the transfer and discharge documents.

Related Documents

Legislation:

- Health Practitioner Regulation National Law (WA) Act 2010
- Nursing and Midwifery Board of Australia (NMBA) Code of Professional Conduct for Nurses March 2018
- Western Australia: Medicines and Poisons Act 2014, Medicine and Poisons Regulations 2016, Pharmacy Act 2010
- Work Health and Safety Act 2020

Policy:

- WA Department of Health Patient Identification Policy 2014
- NMHS Clinical Documentation Policy
- SCGOPHCG Infection Prevention and Control manual
- SCGOPHCG Clinical Handover -Communicating for Safety Policy
- SCGOPHCG Patient Discharge and Discharge Planning Hospital Policy
- SCGOPHCG Detention of Patients and Discharge Against Medical Advice Hospital Policy
- SCGH Patient Identification and Procedure Matching Hospital Policy
- OPH Patient Identification Hospital Policy
- SCGOPHCG Recognising and Responding to Acute Deterioration Hospital Procedure
- SCGH Medication Management Procedure- Patients Own Medicines (POM)
- SCGOPHCG Admission/ Admission of Patients/ Bed allocation
- OPH Hospital Wide Policy Property and Possessions
- SCGOPHCG Hospital Policy – Property (of patient) Management

References

1. The Joanna Briggs Institute. Healthcare Documentation: Abbreviations: [Recommended Practice on the Internet].Joanna Briggs Institute, 2016 [cited 2023 May 01].Available from JBI.(Guideline).
2. Nursing and Midwifery Board of Western Australia. Supervision guidelines for nursing and midwifery. [Internet].20 21[cited 2023 May 01].Available from: <https://www.nursingmidwiferyboard.gov.au/documents/default.aspx?record=WD15%2F17457&dbid=AP&chksum=1f77r9nOSi75OufMD%2BVkHw%3D%3D>
3. Government of Western Australia. Department of Health. Assistant in Nursing Policy. [Internet].2022 [cited 2023 May 01].Available from: https://ww2.health.wa.gov.au/~/_media/Files/Corporate/Policy%20Frameworks/Clinical%20Governance%20Safety%20and%20Quality/Policy/Assistant%20In%20Nursing%20Policy/M-P80-Assistant-In-Nursing-Policy.pdf
4. The Joanna Briggs Institute. Charting by Exception: Patient's care plan:[Evidence Summary on the Internet].Adelaide: Joanna Briggs Institute,2018[cited 2023 May 01].Available from JBI. (Guideline)
5. The Joanna Briggs Institute. Writing Effective Nursing Notes: [Evidence Summary on the Internet]. Adelaide: Joanna Briggs, 2019 [cited 2019 Jul 28].Available from JBI. (Guideline)
6. Australian Commission on Safety and Quality in Healthcare. Communicating for Safety. [Internet].2023 [cited 2023 May 01].Available from: <https://www.safetyandquality.gov.au/standards/nsqhs-standards/communicating-safety-standard>

7. De Groot K, Triemstra M, Francke AL. Quality criteria, instruments, and requirements for nursing documentation: A systematic review of systematic reviews. Journal Advanced Nursing [Internet]. 2019 [cited 2023 May 01]; 1-15. Available from <https://doi.org/10.1111/jan.13919>

Authorisation & Version Control

Policy Sponsor	M Walker, Deputy Nurse Co-Director				
Policy Contact	Nursing Practice Guideline Subcommittee Chair				
Date First Issued:	July 2013	Last Reviewed:	May 2023	Review Date:	May 2026
Version No.	4.0				
Clinical Reviewer:	B Gaston A/Safety and Quality Co-ordinator, J Connolly A CNM Ward 6 PAC OPH D Wheeldon CNM Ward 4 OPH M Collins CNS G62 SCGH				
Executive Sponsor	Executive Sponsor Nursing Practice Guideline Subcommittee				
Endorsed by:	Nursing Practice Guideline Subcommittee			Date:	08/06/2023
Standards Applicable:					

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

8.0 APPENDIX 1 - Weighing Patients

The patient's weight is required based on the need to ensure safe inpatient management and interventions are implemented including:

- The use of appropriate patient equipment i.e. manual handling and diagnostic / interventional equipment that may be weight limited or industry rated
- The use of appropriate manual handling practice to reduce OSH risks for staff
- Ensuring that skin integrity is maintained through the use of appropriate weight based equipment
- Ensuring that patients identified at risk of malnutrition are not further compromised due to weight loss during hospitalisation (an unexplained weight loss is reported to the Medical Officer in charge of care)
- **It is not acceptable** to visually estimate patient weights unless the provision of critical care is required and there are no weighing scales immediately available. The patient or their relatives may provide an estimate of the patient's weight. Thereafter the patient must be weighed as soon as possible.

Weighing equipment available at SCGH and the Hospital Equipment Service:

Weighing a patient		Patient mobility				
		Rest in bed	Hoist but unable to sit in chair	Hoist and sit in chair	Mobile with assist	Mobile independently
Weighing Equipment	Stand on scales	Not applicable	Not applicable	Not applicable	Applicable	Applicable
	Sit on scales	Not applicable	Not applicable	Applicable	Applicable	Can be used if no stand on scales
	Chair scales (SWL 150-220kg) Bariatric chair scales (SWL 500kg)					
	Promed and Arjo Maxi Move Floor Hoist with scales (227-300kg) Gulldman Portable Ceiling hoist (250-255kg dependent on model) Fixed Ceiling hoist (350kg)	Not applicable	Applicable	Applicable	Can be used if appropriate	Can be used if no other option available
	Integrated bed scales in: VersaCare bed (227kg) Compella Bariatric Bed (up to 500kg but therapeutic range for dynamic pressure relieving mattress is 113 kg-455kg)	Applicable	Applicable	Applicable	Applicable	Applicable